

bioSite — Module Submission

Subject: Emily Barr, RN (Operating Room)

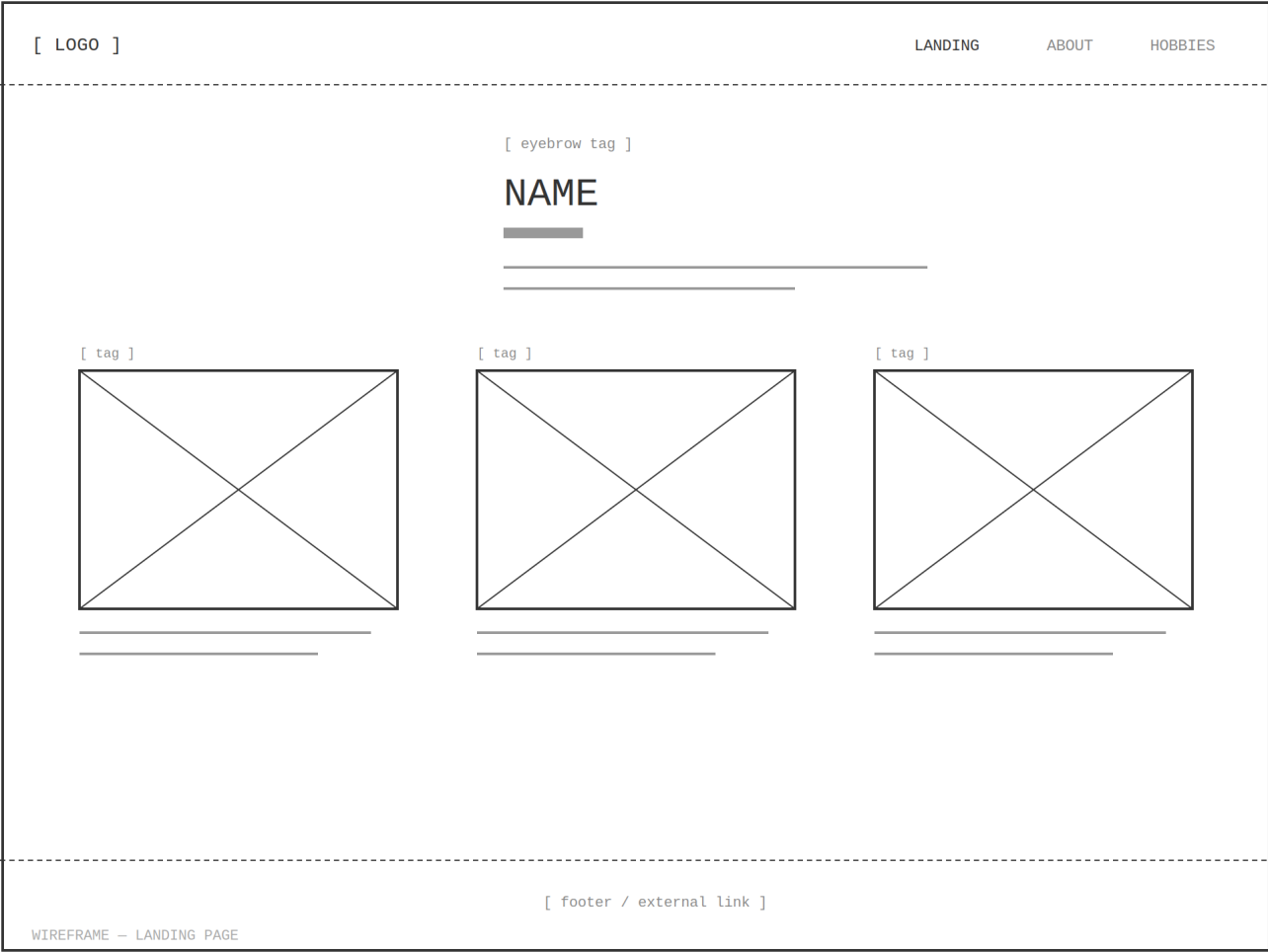
Garvin Stewart

CSD 340 — Web Development with HTML and CSS

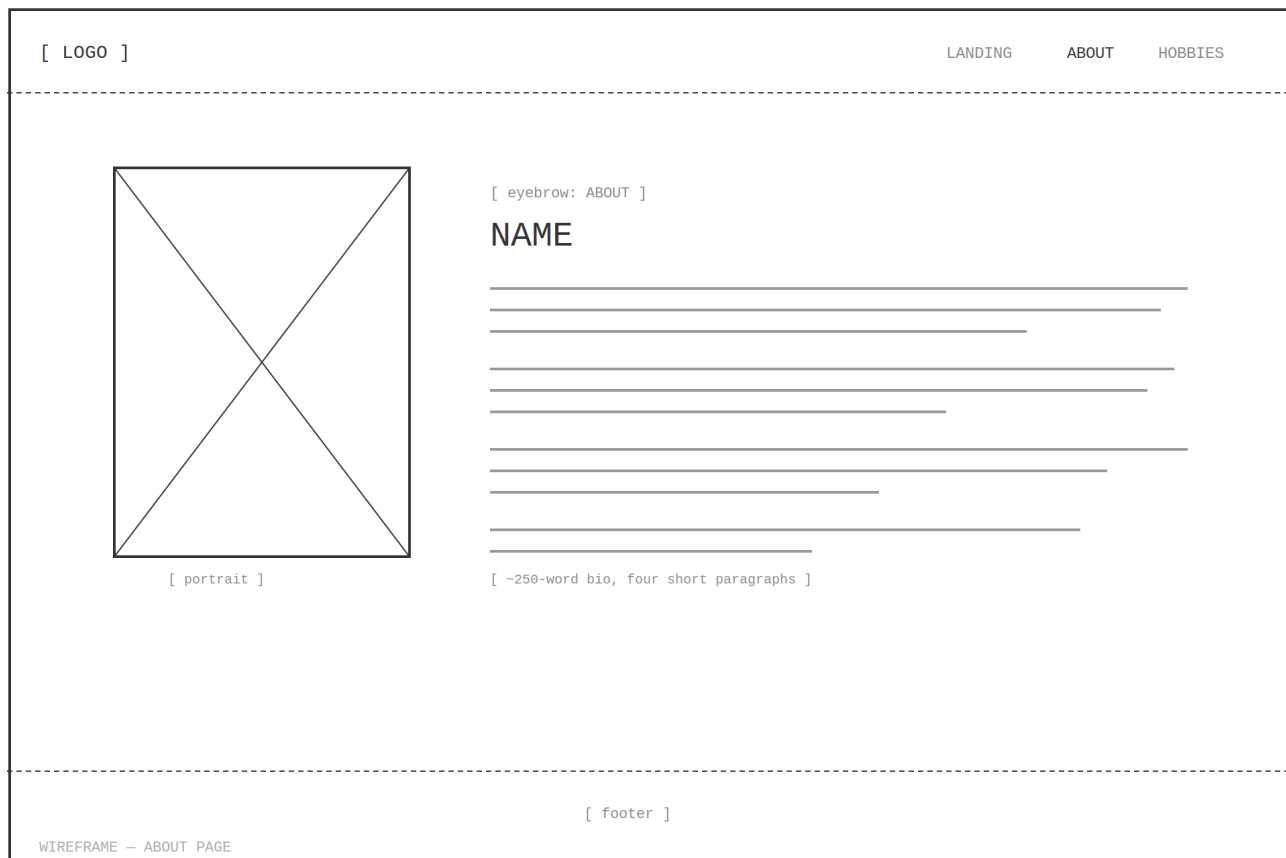
Contents: Wireframes · Front-End Style Guide · Live Page Captures · Subject Interview · Subject Bio

Wireframes

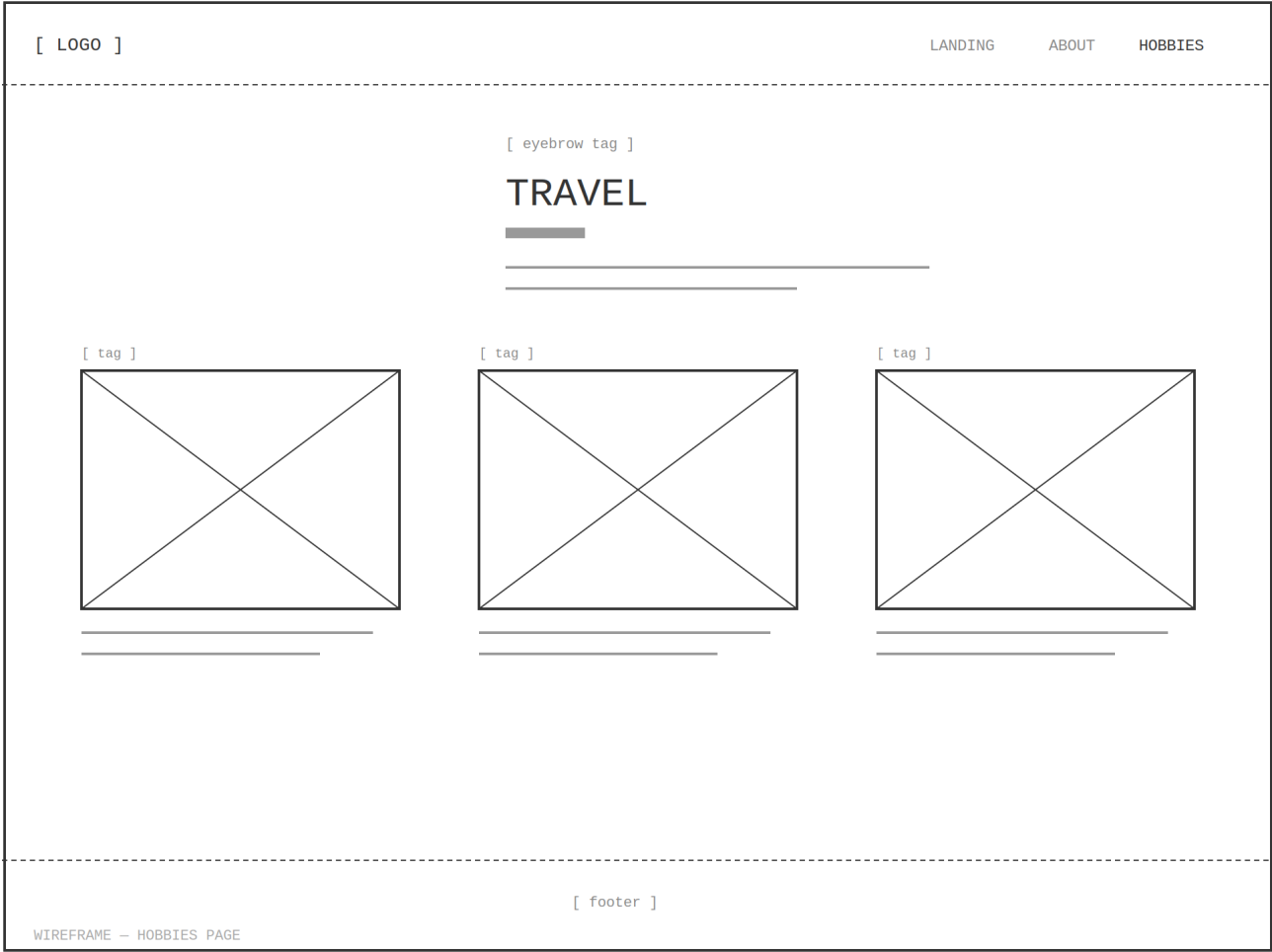
Low-fidelity structure sketches, drawn before the page was coded — one per page.



Landing page — nav, hero, three-card grid, footer.



About page — portrait column, four-paragraph bio column.



Hobbies page — same card-grid component, reused.

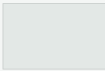
Front-End Style Guide

Rendered directly from site.css — palette, type, spacing, and components.

bioSite — Emily Barr

Every value below is pulled from `site.css`. This page exists so the palette, type, and components stay consistent across Landing, About, and Hobbies — change a token here, and it should change everywhere at once.

Color

					
Clinical White --color-bg / #f4f5f5	Panel --color-panel / #e3e8e6	Ink --color-ink / #1f2b27	Scrub (primary) --color-scrub / #335c4e	Steel --color-steel / #7c8b86	Pop (accent) --color-pop / #d99e2b

Type

DISPLAY — IBM Plex Sans, 700, used once per page for the name/title

Emily Barr

BODY — Inter, 400/600, used for paragraphs and captions

Left the sideline for the operating room — same instinct to help, a different kind of game day. Body copy stays in Inter at 1.6 line-height for readability across longer paragraphs.

TAG / MONO — IBM Plex Mono, used only for small labels (eyebrows, card tags, nav mark)
01 / NURSING SCHOOL

Spacing scale

--space-sm / 0.5rem	
--space-md / 1rem	
--space-lg / 2.5rem	
--space-xl / 5rem	

Signature element

A small dot-pattern swatch stands in for the dachshund-print scrub cap Emily described in her interview — the one deliberately “loud” detail on an otherwise quiet page.
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Components

NAV — reused identically on all three pages

E. BARR, RN	LANDING	ABOUT	HOBBIES
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CARD — one reusable component, used for both Landing milestones and Hobbies trips

01 / TAG



Caption text sits below the image, in Steel, at 0.9rem.

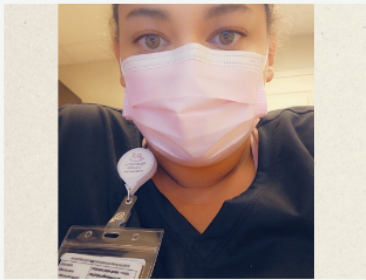
Landing Page (Live)

PERIOPERATIVE RN

Emily Barr

Left the sideline for the operating room — same instinct to help, a different kind of game day.

01 / WHERE IT STARTED



Before nursing school, she was already in scrubs — a CNA in senior care, doing the same job at heart.

02 / NURSING SCHOOL



Graduated while working full time — by her own account, the proudest stretch so far.

03 / IN THE OR



Two years in, and still impossible to mix up with anyone else — dachshund scrub cap included.

Learn more about the field at the [Association of periOperative Registered Nurses](#).

About Page (Live)



ABOUT

Emily Barr

Emily Barr didn't set out to become an operating room nurse — she set out to keep helping people, and nursing turned out to be the better vehicle for it. As an athletic trainer, she'd spent years on sidelines treating injuries in real time; when she made the decision to shift careers, the instinct didn't change, just the room she worked in.

She went through nursing school while working full time, a stretch she still counts among the things she's most proud of — not because it was clean, but because it wasn't. Her first year in the OR was spent precepting: shadowing, repeating, and slowly absorbing the unwritten rules no class teaches, the kind you only learn by being in the room when something goes right and when it doesn't. One early mistake — a specimen sent just as the lab was closing for the day — taught her how unforgiving OR timelines really are, and how much of the job is catching the thing nobody told you to watch for.

Most people have no real picture of what a typical OR day looks like: a standup meeting, an assignment, a string of rooms to prep, then surgery after surgery until the day ends. It's a job that runs on protocol — which is partly why she's particular about the one thing she gets to choose herself: a scrub cap printed with dachshunds, a nod to the one waiting for her at home.

She's not finished. Next up is travel nursing — short-term assignments in operating rooms across the country, the same job, a new room each time.

Hobbies Page (Live)

LIFE OUTSIDE THE OR

Travel

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Off shift, the same energy goes somewhere else — day trips close to home when there's no time off, longer ones when there is.

FALL DAY TRIP



A fall afternoon detour — hay bales and cider, the kind of day trip that still counts as getting away.

ISLAND STOP



A stop somewhere warm — both of them unreadable behind sunglasses, as usual.

SUNSET, ELSEWHERE



Another stamp in the passport — sand, sunset, off the clock.

Subject Interview

Eleven questions, asked directly of the subject.

1. When and how did you first get into nursing?

I wanted a career change, but I still wanted to be doing something built around helping people — which is exactly what I'd been doing as an athletic trainer, just on a sideline instead of in a hospital. Nursing let me keep that same purpose and learn a completely different set of skills to do it.

2. What's a moment in that pursuit you're genuinely proud of?

Graduating nursing school while working a full-time job the entire time I was in it. It wasn't a clean process — there were plenty of nights I questioned whether I'd made the right call — but getting through it without dropping either one is still one of the things I'm proudest of.

3. What's something about it that people who don't do it usually misunderstand?

That nobody actually knows what my day-to-day looks like. People hear “I work in the OR” and picture whatever they've seen on TV, but the real job — the prep, the timing, everything that happens before a surgeon even walks into the room — is something almost nobody outside of it understands.

4. Who or what taught you the most about it?

My preceptor. Nursing school gives you the theory, but the actual rhythm of working in an OR — reading the room, knowing what to grab before anyone asks for it — came from on-the-job training, watching and being watched.

5. What does a typical day look like when you're doing it?

I get in, we run through a quick standup meeting to go over the day's cases, then I get my room assignment. After that it's prepping rooms — making sure everything's sterile and in place — and then it's surgery after surgery until the schedule's done.

6. What's the worst mistake or setback you've had with it, and what did you do after?

There was a specimen I didn't get down to the lab in time — by the time it arrived, they were already closing for the day. It taught me fast how unforgiving OR timelines actually are, and how much of this job is catching the thing nobody specifically told you to watch for.

7. Has it changed how you see yourself, or how others see you?

No.

8. What's something you own related to it that matters to you, and why?

A scrub cap with a dachshund print on it — one of the only places I get to put any personality into what I wear at work. Everything else is regulation: same scrubs, same mask, same shoe covers as everyone else. The cap's the one exception, and it doesn't hurt that the print matches the dog waiting for me at home.

9. Where do you want to be with this in five years?

Still doing travel nursing — moving contract to contract, hospital to hospital, seeing different parts of the country while still doing the job I'm actually trained for. I like the idea of not being tied to one OR for the rest of my career.

10. What would you tell someone just starting out?

Don't be so hard on yourself. There's an enormous amount to learn, and you're going to get things wrong — that's not optional, it's part of how the job actually teaches you. Beating yourself up over every mistake just slows down how fast you improve.

11. Is there a person, place, or community tied to this that's been important to you?

My former coworkers from Novant Health — the people I came up with early on, who were still in the trenches with me while I was figuring out what I was doing.

Subject Bio

Approx. 265 words — written from the interview above, for use on the About page.

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